A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Pushpa Bhaskar
Queens Parade Medical Centre

Dear Dr Bhaskar,

Re: Mr William Au
DOB: 17/07/1961 Phone: 0488688512
Address: 82 Gold Street, COLLINGWOOD, VIC, 3066
Email: William Au
Medicare: 3231275343 / 1

Summary

- **Gastroscopy revealing localised complete intestinal metaplasia.**
- **Colonoscopy revealing a benign polyp.**
- **Combined gastroscopy and colonoscopy surveillance recommended in five years.**
- **No action required by you.**

I had the pleasure of conducting a review with William following his gastroscopy and colonoscopy, which were performed due to a family history of bowel cancer and symptoms of gastroesophageal reflux.

The colonoscopy revealed a benign polyp, which was resected. Given his family history, a five-year surveillance interval is appropriate, the polyp itself was hyperplastic. . The gastroscopy identified an area of inflammation in the stomach. Biopsies of this area confirmed complete intestinal metaplasia. This is currently localised and of a low-risk subtype. I have explained to William that while this can be a precursor to gastric adenocarcinoma, his current findings are stable. We also confirmed that he is negative for *Helicobacter pylori*.

We discussed William's Chinese heritage, which necessitates a more cautious surveillance approach compared to Caucasian populations. We also touched upon his subjective concerns regarding a perceived malodour. He was recently discharged from the Royal Melbourne Hospital where this was managed as a delusional or psychotic symptom. He is also currently being reviewed at the Eye and Ear Hospital for chronic sinusitis.

Impression and Plan

I have recommended a combined gastroscopy and colonoscopy in five years. Although the intestinal metaplasia is low-risk, the combined procedure allows for appropriate surveillance of both the gastric mucosa and the colon. No specific dietary changes or medications are required for the intestinal metaplasia at this stage.

I have counselled William to return sooner should he develop any "red flag" symptoms, specifically early satiety, persistent vomiting, haematemesis, or melaena. If future surveillance shows evidence of progression to dysplasia, we would consider endoscopic resection, but for now, routine monitoring is sufficient.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

AU, WILLIAM SUI HUNG : SVH 655012, SVP 425397, NYCH 42991

DOB: 17/07/1961 Age: 64 years

82 Gold St, Collingwood 3066

Results from 11/04/2026 ..... Episode# 26P275611 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009379 **CLINICAL NOTES** Gas = ? IM. Rectal polyp= HP **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric bx". Five mucosal fragments measuring 2-5 mm. A1. 2. Specimen labelled "Rectal polyp". Two tan polypoid fragments 2 x 2 x 1 mm and 3 x 2 x 1 mm. Processed whole. A1. **MICROSCOPIC DESCRIPTION** 1. The sections show non-specialised and transitional gastric mucosa, all of which show partial involvement by complete intestinal metaplasia. There is a mild increase in chronic inflammatory cells within the lamina propria including scattered plasma cells. No active or granulomatous inflammation is seen and Helicobacter pylori like organisms are not identified. There is no dysplasia or malignancy. 2. The sections show polypoid colonic mucosa with focal serrated luminal architecture, not extending to crypt bases. There is no dysplasia or malignancy. **PATHOLOGIST'S OPINION** **1. Gastric biopsy: Complete intestinal metaplasia. 2. Rectal polyp: Hyperplastic polyp.** Pathologist: Dr Rohini Joshi

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